Gemcitabine and cisplatin induction before chemoradiotherapy

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Later chemoradiotherapy: separate prescription

Active medicines:

Emetogenic risk: Minimal

The later chemoradiotherapy regimen needs its own prescription and support plan; no induction medicines carry over.

Additional phase-specific support

- Gemcitabine runs over 30 minutes, followed on Day 1 by cisplatin over 1 hour. Prescribed fluids before and after cisplatin lengthen the visit. Day 8 is gemcitabine alone.
- Day 1: full blood count, kidney and liver tests. Day 8: full blood count. Audiology is arranged when clinically indicated; extra tests depend on symptoms and results.
- The NCCP plan starts chemoradiotherapy 21–28 days after Day 1 of the last induction cycle, subject to review and recovery. This is measured from the first day, not from Day 8 or the end of that cycle. The later regimen has its own prescription.
- Use current local supportive-care policy; review every medicine before prescribing.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____